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HAVEN PAEDIATRIC CENTRE

The People Work at Haven

An engagement guide for the people workstream

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CIRCULATION
Internal to Consult for Africa. Not for the client.

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A letter before you start

Dear Abigail,

You are taking on the part of the Haven engagement that decides whether the culture work lasts. Four pieces of it are yours: getting every person into the right post, writing down what each post is, giving every person a proper contract, and agreeing what each of them is working towards. They sound like paperwork. At Haven they are the culture, because they are what tells twenty people what the hospital expects of them and what they can expect back.

You will do this alongside Dr Omotanwa Gbadebo, who goes by Dr Tanwa and is running operations at Haven on our behalf. She is in the building, she holds the roster and the establishment, and she has the clinical standing with the nurses and doctors that neither of us has. You hold the method, the drafts, the records and the follow-through. Neither half works without the other.

Nobody expects you to arrive as an HR specialist. I made you a promise earlier this month that anything new would come to you with an example of one that is already right, and this guide keeps it. Section 10 has a finished job description, a finished set of objectives, a contract checklist and a personnel file checklist. Copy them. Change what does not fit. Ask about anything that is unclear.

Read section 2 before you speak to anybody at Haven. The conversations in this workstream are more personal than any others on the engagement, because they touch people's jobs and their pay, and the rules in that section protect them as well as us.

The honest warning is about fear. When a consultancy arrives and starts asking people what they actually do, some of them will assume it ends with somebody losing a job. That assumption is the single biggest risk to this work. The way you open each conversation, and the care you take with what people tell you, will decide whether they trust the result. Section 9 is about exactly that.

Mark anything blocked the moment you are unsure, and call me about anything you think you should already know. I would much rather be asked twice than find out later that you were guessing.

Dr Debo Odulana

Founding Partner, Consult for Africa

01 Where you sit

Haven is a client. It is the employer of every person you will work with, which means we draft and Haven decides. No contract, job description or objective reaches a member of staff in Consult for Africa's name. Each one goes out from Haven, approved by the people at Haven who own that decision.

Who	What they hold on this work
You	The method, every draft, the conversations with staff, the tracker and the files. You keep the four pieces moving and you know where each person is in each of them.
Dr Tanwa	The establishment: how many posts the hospital needs and on which shifts. Content review of every draft. Sits in on conversations with clinical staff.
Dr Odedina	Medical Director. Signs off every clinical job description and every clinical objective. Address him as Dr Odedina, always.
The founders	Approve the establishment, pay, benefits and the contract template. A founder or their authorised signatory signs each contract.
Me	The board, anything to do with money, anybody's job being at risk, and anything in section 2.
Ezinne	Remains your line manager for your office work. Keep her copied on your weekly note so she knows where your time is going.

Bring these to me before you act, every time: any question about a person's pay, any suggestion that a post should go or a person should leave, any disciplinary matter, any change to contract terms, the benefits question, and anything in section 2.

02 Read this first. The sensitivities

The incident, and how we hold it

In May 2026 Haven lost a child who was referred in already critically ill. The loss is not attributable to the facility, and it is what prompted the leadership to call us.

We do not raise it, in writing or in conversation, ever. If a member of staff raises it with you, listen with care, do not ask questions about it, and move the conversation on gently. Never write anything that connects it to a role, a gap in staffing or a person. If you find yourself about to, stop and call me.

Pay, and the adjustment round

Earlier this year Haven ran a salary adjustment round that staff read as unfair, because some received much larger increases than others and nobody explained why. It is still a live grievance. Expect it to come up in your conversations.

You do not comment on it, defend it or criticise it. Write down what the person says in their own words, thank them, and tell them it will go to the people who can answer it. Never discuss one person's pay with another, never confirm what somebody else earns, and never send salary figures by WhatsApp or in a group email. Payroll figures come from Haven's Head of Finance when a contract needs them, and they go nowhere else.

Nobody's job is ours to promise or threaten

You will be asked, directly, whether somebody's job is safe. The honest answer is that this work is about writing down properly what each person does and making sure the hospital has the right posts for the work in front of it. Decisions about posts belong to Haven's founders. Do not promise anybody their job is safe and do not hint that it is not. If anybody presses, tell them you will make sure the question reaches the right person, and bring it to me.

Personal data

Contracts, ID copies, licences, bank details and pay are personal data under the Nigeria Data Protection Act 2023, and health information is sensitive data with stricter rules. Before you scan or upload the first signed document, ask me where the files are to be kept. Not on WhatsApp, not in email threads, not in a personal Google Drive and not on your own laptop.

The relationship

I was invited to Haven's board from inception but do not sit on it. Dr Shakirah Saliu is a former co-founder of mine. Know it, never imply leverage from it, and never discuss the commercial terms of our engagement with anybody at Haven.

03 What we found about Haven's people

CONFIRMED FINDINGS	PERFORMANCE MANAGEMENT	ON THE ORGANOGRAM
7 of 12	**1 of 5**	**6 posts**
were about people systems	no objectives, no appraisals, no review cycle	with nobody in them

The August audit found a hospital with a warm, capable team and almost none of the structure that lets a team like that grow. Staff were asked to write their own performance indicators. Nobody has had a formal appraisal. Induction is learning by watching. The organogram names a matron, a head of clinicals, a head of laboratory, a billing officer, a facility officer and an insured revenue officer, and none of those posts exists on the payroll or the roster.

The roster showed what happens when the posts on paper and the work on the floor drift apart. People cover gaps that are not theirs to cover, with good intentions, because there is nobody else. The audit found a pharmacy technician rostered to nursing shifts. That is a fault in the establishment and the roster. It is not a fault in her, and it is never to be raised with her as though it were.

The good news is large. Colleagues treat each other well, sixteen of seventeen staff rated the safety of care very good or excellent, and role clarity scored high even without written objectives. People mostly know what their job is. Most of your job descriptions will be a record of good practice that already exists rather than an attempt to change it.

The loudest staff request in the survey was health cover and a pension. That is a decision for the founders and it is on their list. It matters to you because a contract has to state the pension position, so the contracts cannot be finished until that decision is made. Section 7 explains.

The twenty people on the list

Department	Posts on the current list
Medical	Chief Medical Director, three Medical Officers
Nursing	Senior Registered Nurse, five Registered Nurses
Administration	Head of Admin, Operations and Business Development; Head of Customer Service; Head of Finance; Admin and Billing Officer; Business Development
Laboratory	Medical Laboratory Scientist
Pharmacy	Two Pharmacy Technicians
Domestic	Two Attendants

Treat the list as a starting point. Confirm every name and title against Haven's payroll before you rely on it, because the audit found the roster and reality did not always agree.

04 The four pieces, and why the order matters

The four pieces have to run in order, because each one is built on the one before it. You cannot write a contract for a post that has not been described, and you cannot set objectives for a role nobody has agreed.

Order	The piece	What it produces	Who approves
1	Staff realignment	An establishment table: every post the hospital needs, and who holds each one	Dr Tanwa, then the founders
2	Job descriptions	One page per post, dated and signed off	Dr Odedina for clinical posts, Dr Tanwa for the rest
3	New contracts	A contract per person, with their job description attached	The founders approve the template, then sign each one
4	Objectives	Five objectives per department, then five per person	Department head, then Dr Tanwa

The timetable

Weeks	What happens
26 Oct to 6 Nov	Realignment. Twenty conversations, the establishment table drafted and reviewed with Dr Tanwa. To the founders by Friday 6 November.
9 Nov to 27 Nov	Job descriptions drafted with each department, reviewed and signed off. The contract template goes to Haven's lawyer by 13 November.
30 Nov to 18 Dec	Contracts issued and explained one to one, once the founders have approved the template and decided the pension and benefits position. Department objectives agreed with each head by 11 December.
20 Dec to 4 Jan	Nothing new. The hospital is at its busiest, and anything introduced in that window gets blamed for whatever goes wrong in it.
From 5 Jan	Personal objectives set in each person's first one to one of the year.

This runs alongside the culture sprints, and your chase work in the culture plan carries on. Use the same visits for both wherever you can, so that nobody is pulled off shift twice for something that could have been done once.

05 Staff realignment

Realignment means making the posts on paper match the work the hospital actually needs, and putting each person in the post that matches what they do. It is not a restructuring, and you should not use the word realignment with staff. To them it is "making sure everyone's role is written down properly".

Establishment is not headcount

Establishment is the number of posts the hospital needs to run safely, by role and by shift. Headcount is the number of people employed. Haven has been running with fewer posts than its service needs, and the gap has been filled by people stretching across roles. Dr Tanwa works out the establishment from the beds, the shifts and the volume. You work out who is doing what today.

How to do it

Start from three lists: the organogram, the payroll and the last two months of rosters. Then sit with each person for fifteen minutes on shift, ideally with their department head's knowledge, and ask them to walk you through a normal day and a bad day. Write what they tell you in their words.

For every person, fill in one row of the realignment table.

Column	What goes in it
Name	As on the payroll
Title on paper	From the payroll or offer letter
What they actually do	From the conversation and the roster
Hours and shifts actually worked	From the roster
Reports to, in practice	Who they actually go to, which is often not who the organogram says
Registration held	NMCN, MDCN, MLSCN or PCN, with the expiry date
Proposed post	Left blank until Dr Tanwa has drafted the establishment
Note	Anything that needs a decision. Factual, never a judgement about the person

When the table is complete, every row falls into one of four groups: the person and the post match; the person is doing work that belongs to a different post; the person is covering two posts; or there is a post with nobody in it. The last group is the vacancy list for the founders. The middle two are where the conversations in section 9 matter most.

What you never do

Never tell a person their proposed post before the founders have approved the table. Never put a judgement about performance in the table: it describes posts and work, not how well anybody does it. If you find somebody working outside the scope of their registration, take it to Dr Tanwa that day, privately. It is a clinical safety matter for her and Dr Odedina to resolve through the roster, and the person concerned should only ever hear about it as a change to their shifts.

06 Job descriptions

One page per post, not per person. Five registered nurses share one Registered Nurse job description.

How to write them

With the people who do the job, never at them. Use what you heard in the realignment conversations, draft it, then sit with the department head and one person in the post and read it through together. Most of the time the existing practice is sound and nobody has written it down. Your draft is a record, not a reform. Where something in the draft surprises them, you have usually found a constraint nobody told you about, so write that down too.

Every job description has the same nine parts, in this order: post title and department; reports to; who reports to this post; the purpose of the post in one sentence; six to eight main responsibilities, each starting with a verb; qualifications, registration and experience required; the working pattern and shifts; how performance is reviewed; and the date, version number and who approved it.

Keep the responsibilities to the work the post does every week. A list of twenty duties tells nobody anything, and "any other duties as assigned" belongs once, at the end, not throughout.

The registration line matters more in a hospital than anywhere else. Nurses are registered with the Nursing and Midwifery Council of Nigeria, doctors with the Medical and Dental Council of Nigeria, laboratory scientists with the Medical Laboratory Science Council of Nigeria, and pharmacists and pharmacy technicians with the Pharmacists Council of Nigeria. Each job description says which registration the post requires and that it must be kept current. The job description is where the clinical scope of a post is fixed in writing, which is why Dr Odedina signs every clinical one.

Approval

Clinical job descriptions go to Dr Odedina through Dr Tanwa. Non-clinical ones go to Dr Tanwa. All of them then go to the founders as one set alongside the establishment. Nothing reaches a member of staff until it is approved, dated and numbered. An undated job description is a rumour.

The worked example in section 10 is the one to copy.

07 New contracts

Why this needs care

Nigerian employment law requires an employer to give every worker written terms of employment within three months of starting. Most of Haven's team either have no written contract or have an offer letter that does not cover what a contract should. Putting that right protects them and protects Haven, and it is one of the clearest signals the founders can send that they take their staff seriously.

It also carries more legal weight than anything else in this workstream. So the template is drafted once, reviewed once by a Nigerian employment lawyer that Haven appoints, approved by the founders, and then used for everybody. You do not draft a contract from scratch for any individual, and you never change a clause for one person without my written agreement.

The rules that protect staff

A new contract must not make anybody worse off than they are today on pay, hours, leave or any benefit they already have, unless they agree to it in writing after it has been explained to them. If any draft would do that, stop and bring it to me.

The start date in the contract is the date the person first joined Haven, not the date they sign. A new contract must never reset someone's length of service, because length of service is what notice periods and many entitlements are counted from.

Probation applies to new joiners only. Somebody who has worked at Haven for a year is not put on probation by a new piece of paper.

Pension and benefits

The Pension Reform Act 2014 makes a contributory pension compulsory for an employer of Haven's size, with a minimum employer contribution of ten per cent. That makes pension a legal question as well as the staff's loudest request, and the contract has to state the position. Health cover is not compulsory in the same way, but the contract states whatever the founders decide. Neither is yours to answer. I will take both to the founders, and the contracts wait for their decision rather than going out silent on it.

Issuing a contract

Each contract is handed over in person, one to one, with the job description attached. You explain what has changed and what has not. The person takes it away for at least three working days, can ask any question, and can ask a family member or adviser to read it. Then they sign two copies, a founder countersigns, the person keeps one and the other goes into their file.

Record each step in the tracker: drafted, approved, explained, signed by the person, countersigned, filed. A contract that has been signed but not filed does not exist when somebody needs it.

08 Objectives

The audit's six-month test is simple. Every member of staff can state their five objectives and has had at least two documented conversations about them. At day sixty I will ask three people at random what their objectives are.

Departments first, then people

Department objectives come first, written by the department head with Dr Tanwa and with your help on the wording. Personal objectives are then drawn from the department's, so that each person can see how their work adds up to their team's.

What a good objective looks like

Five or fewer. Each one names a measure, a number, where the number comes from and when it is checked. Each one is something the person can actually affect. Each one ties to something in the growth plan or the culture plan, so it is clear why it matters. If a nurse cannot explain an objective to a colleague in one sentence, it is not finished.

Where the data does not exist yet, the first objective is to start recording it. That is a perfectly good objective for the first quarter.

The line you must not cross

Never set an objective for a doctor or nurse based on the number of admissions, length of stay or tests ordered. Clinical decisions have to be kept separate from targets, because an objective that rewards volume puts patients at risk, whatever the intention. Clinical objectives are about quality and reliability: documentation complete at the point of care, standards followed, checks done, incidents reported, resuscitation training current. Dr Odedina signs every one of them.

Objectives are not pay yet. Do not tell anybody that money attaches to them. Whether and how the founders link reward to performance is a later decision, and a promise made early that is not kept would

undo everything the objectives are for.

The conversations that follow

An objective nobody talks about is a document in a drawer. Each person gets a fifteen-minute monthly one to one with their department head and a quarterly review. Your job is to schedule them, make sure a two-line note of each is kept, and chase the ones that slip, which is the same chase you already run for the culture plan.

09 Talking to staff

Every piece of this work starts with a conversation, and how the conversation goes decides how the paperwork is received.

> *"I'm Abigail, I work with Dr Tanwa. We're making sure everybody's role at Haven is written down properly, so everyone knows what is expected of them and what they can expect back. Can you walk me through a normal shift for you? And then a hard one?"*

Open the same way with everybody, so nobody feels singled out. Talk to people on shift, at a moment their department head has agreed, and never pull a nurse away from a patient to do it.

Listen more than you talk. Write down what they say in their words, not your summary. Tell them who will see your notes: Dr Tanwa and me, and nobody else at Haven in a form that identifies them.

If somebody is anxious about their job, say what is true: the work is about writing roles down properly, and decisions about posts sit with the founders. Then bring their concern to me so it is not lost.

If somebody raises a grievance about pay, the adjustment round, a colleague or a manager, thank them, write it down, and tell them it will reach somebody who can answer it. Do not agree, disagree, or promise an outcome.

If somebody raises the May loss, follow section 2.

When a conversation goes badly, stop, thank the person, and tell Dr Tanwa the same day. There will always be another day to finish.

10 Examples to copy

A finished job description: Registered Nurse

POST	Registered Nurse
DEPARTMENT	Nursing
REPORTS TO	Senior Registered Nurse
REPORTS TO THIS POST	None
VERSION	Draft 1, for review by Dr Odedina

Purpose of the post. To deliver safe, documented nursing care to children and newborns on the ward, in the neonatal unit and in outpatient triage, on the shifts set by the roster.

Main responsibilities

- Assess, plan and deliver nursing care to the patients allocated on each shift, within the standards and protocols approved by the Medical Director.
- Give medicines as prescribed, and record each dose on the drug chart at the time it is given.
- Hand over every patient at each shift change using the agreed handover format, and record that the handover took place.
- Complete the crash trolley check at the start of every shift and sign the checklist.
- Recognise a deteriorating patient and escalate to the medical officer on duty without delay, following the escalation pathway.
- Report every incident and near miss through the hospital's reporting form on the same shift.
- Keep the nursing record complete, legible and signed at the point of care.
- Support parents with clear information and refer any question outside nursing scope to the medical team.

Qualifications and registration. Registered Nurse with current registration with the Nursing and Midwifery Council of Nigeria, kept current throughout employment. Current basic life support certification, with paediatric and neonatal resuscitation training completed within six months of joining. Paediatric or neonatal experience preferred.

Working pattern. Shifts as set by the published roster, including nights, weekends and public holidays. Swaps agreed in advance with the Senior Registered Nurse and recorded.

How performance is reviewed. Against the individual objectives agreed each year, in a monthly one to one with the Senior Registered Nurse and a quarterly review.

Approved by. Dr Odedina, Medical Director. Date. Version.

Department objectives: Nursing, first quarter

Objective	Measure	Where the number comes from	Checked
Every handover recorded	Share of shift changes with a completed handover record. Target: all of them	The handover log	Weekly by the Senior Registered Nurse
The crash trolley ready every shift	Share of shifts with a signed trolley check. Target: all of them	The trolley checklist	Weekly
Drug charts complete	Share of sampled charts with every dose signed at the time. Target: 95 per cent	Ten charts sampled each month with Dr Odedina	Monthly
Speaking up is normal	Incident and near miss reports from nursing each month. Target: rising, and every one closed out	The incident register	Monthly
Resuscitation training current	Share of nurses with current basic life support. Target: all of them by March	The training register	Monthly

The fourth objective is the one people misread. A rise in reports is good news: it means people feel safe to speak up. Say so whenever you share the numbers.

Department objectives: Front desk and customer service, first quarter

Objective	Measure	Where the number comes from	Checked
Every visit registered and billed the same day	Visits billed against visits seen. Target: all of them	The daily three-count reconciliation	Daily
HMO authorisation before service	Share of insured visits with authorisation recorded before treatment, except emergencies. Target: 95 per cent	The billing record	Weekly
Deposits collected on admission	Share of self-pay admissions with the deposit taken. Target: all of them	The billing record	Weekly
Complaints answered	Share of complaints logged and answered within two working days. Target: all of them	The complaints log	Weekly
Parents would recommend Haven	Average score on the patient survey. Target: hold or improve	The patient survey	Monthly

Contract checklist

Every contract issued from the approved template must have each of these filled in and checked before it goes to a founder for signature.

- Haven's full registered name and address as the employer
- The person's full legal name, as on their ID
- Post title, exactly matching the approved job description, which is attached
- Original start date at Haven, not the date of signing
- Probation stated only for joiners in the last six months, and not otherwise
- Pay as confirmed in writing by the Head of Finance, unchanged unless the founders approved a change in writing
- Hours and shift pattern, matching the job description
- Annual leave, sick leave and public holidays
- Pension and any benefits, exactly as the founders decided
- Notice period on each side
- Confidentiality and patient data
- Reference to the staff handbook, code of conduct, disciplinary and grievance procedures
- Registration requirement, for clinical posts
- Nothing in the contract leaves the person worse off than today without their written agreement

Personnel file checklist

One file per person, kept where I confirm, holding:

- Signed contract, countersigned
- Approved job description, the version they were given
- Copy of ID
- Copies of qualification certificates
- Current practising licence, with the expiry date recorded in the tracker
- Resuscitation and other training certificates
- Next of kin and emergency contact
- Pension fund administrator details, once the pension position is settled
- Signed acknowledgement of each protocol that applies to the post
- Objectives for the year, and the notes of each review

The tracker

One row per person, one column per step: realignment conversation held; proposed post approved; job description approved; contract drafted; contract explained; contract signed; contract countersigned; file complete; licence expiry; department objectives agreed; personal objectives agreed; last one to one. Dr Tanwa and I should be able to open it at any moment and see exactly where everybody is.

11 How we work

Send Dr Tanwa and me a note every Friday, copying Ezinne, kept to a page: what moved this week, what is stuck and why, and what you need decided. Every draft goes to Dr Tanwa first. Anything going to the founders comes through me.

You will get a written note back on every piece of work you send, saying what was right and what to change. If a week goes by without one, chase me for it.

Mark something blocked the moment you are unsure rather than when it is late. A question asked on a Tuesday costs nothing. A contract issued wrongly is very hard to take back.

And tell me where the plan is wrong. You and Dr Tanwa will be in the building talking to twenty people, and I will not. If the order, the timetable or any example in this guide does not fit what you are seeing, say so, say why, and bring what you would do instead.

Welcome to Haven.